

MARCH → where each step lives in the QRCs

AFTTP 3-4 never prints the word “MARCH.” Its own sequence is **QRC 19.1 (casualty evaluation)**. This page lines the standard TCCC acronym up with the cards that cover each letter, using 19.1’s own cross-references.

M MASSIVE HEMORRHAGE	Stop life-threatening bleeding first. QRC 19.3 C-A-T tourniquet, Care Under Fire · QRC 19.4 direct pressure, pressure points, combat gauze (Tactical Field Care) · QRC 19.5 C-A-T, Tactical Field Care. Tourniquet “high and tight,” never loosen it, mark “T” + time.
A AIRWAY	QRC 19.6 Airway: head-tilt/chin-lift (no neck/back injury) or jaw-thrust (possible injury) · QRC 19.7 NPA insertion: measure nostril to earlobe, lubricate with water-based only, bevel toward the septum.
R RESPIRATION	QRC 19.8 Sucking chest wound — ensure the casualty is breathing. (19.1 step 3.)
C CIRCULATION	Recheck bleeding (19.4, 19.5) and treat shock: QRC 19.9 Treat for shock — confusion; pale, cool, clammy skin; fast shallow breathing; rapid pulse. Keep warm, no food or drink. Note: 19.1 itself uses the word “Circulation” for the bleeding step.
H HEAD INJURY / HYPOTHERMIA	QRC 19.10 Head/neck/spinal injury (immobilize) · 19.1 step 5, minimize environmental exposure — see Chapter 23, QRC 23.2–23.3 cold stress and frostbite.

After MARCH (the rest of 19.1): open wounds **19.4** · abdominal wound **19.11** · eye injuries **19.12** · sprains/strains/fractures **19.13** · burns **19.14** → document on **DD Form 1380** → evacuate, **MEDEVAC 9-line, QRC 20.1**.

Under fire (QRC 19.2): return fire as directed; casualty stays engaged or moves to cover / self-aid or plays dead; if a limb is bleeding badly, **tourniquet only**, then evacuate. Full MARCH waits until you are out of the fire.

Letter-to-card mapping is a cross-reference built from QRC 19.1’s own “see QRC” references. Check the exact wording your cadre expects for the graded run.

QRC 19.1 GENERAL SELF-AID AND BUDDY CARE (CASUALTY EVALUATION)

Determine if Care Under Fire, see QRC 19.2 or
Tactical Field Care, see QRC 19.4

Always Treat for Shock, see QRC 19.9

Use Personal Protective Equipment (Gloves)

1. Stop any life-threatening bleeds (Circulation, see QRC 19.3, 19.5, and 19.8)
2. Establish an airway (Airway, see QRC 19.6)
3. Ensure casualty is breathing (Sucking Chest Wound, see QRC 19.8)
4. Prevent further disability:
 - Immobilize head/neck/back injuries (see QRC 19.10)
 - Place dressings on any other open wounds (see QRC 19.4)
 - Splint limb injuries (see QRC 19.13)
 - Treat abdominal wounds (see QRC 19.11)
 - Treat eye injuries (see QRC 19.12)
 - Treat burns (see QRC 19.14)
5. Minimize further environmental exposure (see Chapter 23)
6. Document all treatment on DD Form 1380
7. Evacuate as needed (see Chapter 20)

QRC 19.2 CARE UNDER FIRE

Return Fire As Directed/Required Before Providing Any Medical Treatment

1. Direct casualty to remain engaged as a combatant (if possible)
2. Direct casualty to move to cover and apply self-aid, if able
3. If casualty cannot return fire, have casualty “play dead” until enemy fire is suppressed and rescue plan is developed and executed
4. If the casualty has life-threatening bleeding coming from a limb, apply a tourniquet on the limb that is bleeding
5. Evacuate

QRC 19.3 COMBAT APPLICATION TOURNIQUET (C-A-T®) -CARE UNDER FIRE-

Always Treat for Shock, see QRC 19.9

Use Personal Protective Equipment (Gloves)

WARNING: Do not use wire, shoestring, or anything less than 1 inch wide.

WARNING: Never loosen or remove tourniquet once it is applied.

1. Expose wound, and place C-A-T® around extremity as high and tight as possible
2. Route strap through friction adapter buckle
3. Pull free end of band tight, and secure it onto itself using the velcro
4. Twist the windlass rod until bleeding stops

NOTE: If you can twist the windlass rod more than three times, start over and ensure that the band is pulled tight against the extremity before twisting the windlass rod.

5. Insert rod into windlass clip
6. Pull windlass strap tight over windlass clip and band to prevent the windlass rod from unwinding
7. Mark the patient's head with the letter “T” and time that the tourniquet was applied, also annotate the date/time/location of the C-A-T® on the casualty card
8. If needed, apply second tourniquet below initial tourniquet if bleeding has restarted or not stopped

QRC 19.4 CONTROL BLEEDING -TACTICAL FIELD CARE-

Always Treat for Shock, see QRC 19.9

Use Personal Protective Equipment (Gloves)

1. Apply direct pressure with hand and use dressing if available
2. If bleeding has not stopped with direct pressure, use pressure points with elevation to control the bleeding
3. If available, use combat gauze

NOTE: If the bleed is coming from groin or armpit, where tourniquet cannot reach, use combat gauze with direct pressure only after you move to tactical field care phase.

4. If bleeding has not stopped with direct pressure, pressure points, or elevation, apply a tourniquet to stop bleeding in the tactical field care/conventional phase.
5. Document all treatment on DD Form 1380.
6. Evacuate as needed

**QRC 19.5 COMBAT APPLICATION TOURNIQUET
(C-A-T®) -TACTICAL FIELD CARE-**

WARNING: *Do not* use wire, shoestring, or anything less than 1 inch wide.

WARNING: *Never loosen or remove* tourniquet once it is applied.

NOTE: Leave tourniquet area exposed for quick visual reference.

- Expose wound, and place C-A-T® around extremity 2-4 inches if possible
- Route strap through friction adapter buckle
- Pull free end of band tight, and secure it onto itself using the Velcro®
- Twist the windlass rod until bleeding stops

NOTE: If you can twist the windlass rod more than three times, start over and ensure that the band is pulled tight against the extremity before twisting the windlass rod.

- Insert rod into windlass clip
- Pull windlass strap tight over windlass clip and band to prevent the windlass rod from unwinding
- Mark the patient's head with the letter "T" and time that the tourniquet was applied and annotate the date/time/location of the C-A-T® on the casualty card
- If needed, apply second tourniquet below initial tourniquet if bleeding has not stopped or restarted

QRC 19.6 AIRWAY

Determine if Care Under Fire, see QRC 19.2 or
Tactical Field Care, see QRC 19.4
Always Treat for Shock, see QRC 19.9
Use Personal Protective Equipment (Gloves)

HEAD-TILT/CHIN-LIFT METHOD

(No head/neck/back injuries)

1. Place one hand on the casualty's forehead
2. Place fingertips of other hand under bony aspect of casualty's chin
CAUTION: Do not press on the soft area under the chin, doing so may block the casualty's airway.
3. Gently tilt casualty's head by applying downward pressure on forehead while lifting chin upward
4. The mouth should open; use your thumb to depress the casualty's lower lip slightly to keep the mouth open

JAW-THRUST METHOD

(Possible head/neck/back injuries)

1. Take a position at the top of the casualty's head, resting your elbows on the ground on either side of the head
2. Reach forward and place one hand on each side of the casualty's lower jaw, at the angle (just below the hinge) of the jaw
3. Stabilize the casualty's head between your forearms
4. Using your fingers, gently push the lower jaw forward to open the airway
5. The thumbs can be used to depress the casualty's lower lip slightly to keep the mouth open

CAUTION: Do not tilt or rotate the casualty's head, doing so may cause further injury to the spinal cord.

6. Immobilize the casualty using any means possible (e.g., cervical collar, rolled up towels, boots filled with sand) (QRC 19.10)

QRC 19.7 NASOPHARYNGEAL AIRWAY (NPA) INSERTION

Determine if Care Under Fire, see QRC 19.2 or
Tactical Field Care, see QRC 19.4

Always Treat for Shock, see QRC 19.9

Use Personal Protective Equipment (Gloves)

1. Measure the NPA length from the nostril to the earlobe

NOTE: Only one size NPA is in the JFAK.

2. If the NPA is too long, cut it to the correct length; when cutting, ensure you cut at the tip at an angle (bevel)

3. Lubricate the NPA with water, the casualty's saliva or a water-based lubricant

CAUTION: *Do not* use petroleum jelly or any other non-water base lubricant as they will damage the tissue in the nasal cavity and increase the risk of infection.

4. Face the bevel portion (angle) towards the center of the nose

5. Gently push the nose up and insert the NPA straight back (not upwards) into the nostril

6. Advance the NPA until the flange rest firmly against the outside of the nose

CAUTION: If you experience difficulty advancing the NPA, pull it out and try the other nostril.

7. Document all treatment on DD Form 1380

8. Evacuate as needed

QRC 19.8 SUCKING CHEST WOUND

Determine if Care Under Fire, see QRC 19.2 or
Tactical Field Care, see QRC 19.4
Always Treat for Shock, see QRC 19.9
Use Personal Protective Equipment (Gloves)

SYMPTOMS

- Sucking noise from chest
- Frothy (foamy) red or pink blood from wound
- Difficulty or pain with breathing

TREATMENT

1. Expose chest to see entry/exit wound
2. Wipe off any debris of fluids prior to dressing the wound
3. Have casualty exhale, then place dressing
4. Cover wound with a chest seal from the JFAK or an improvised occlusive (airtight) dressing (such as an MRE package wrapper, plastic from field dressing, or duct tape)

NOTE: Improvised occlusive dressings must be 2 inches bigger than wound.

5. Tape down all four sides of dressing extending 2 inches past the occlusive dressing
6. Determine if there is an exit wound

NOTE: When casualty is being turned to look for or treat exit wound, turn casualty on affected side and apply occlusive dressing on the exit wound the same way.

7. Allow casualty to assume position of comfort where it is easiest for them to breathe
8. Document all treatment on DD Form 1380
9. Evacuate as needed

QRC 19.9 TREAT FOR SHOCK

SYMPTOMS

- Confusion
- Sweaty, pale, cool skin (clammy skin)
- Breathing shallow, labored, and rapid
- Rapid pulse

TREATMENT

1. Stop any life-threatening bleeds (Circulation, see QRC 19.3, 19.5, and 19.8)
2. Keep airway open (Airway, see QRC 19.6)
3. Treat any other major wounds
4. Position the casualty on their back or in a position that allows them to breathe. Position an unconscious casualty in the recovery position so fluids can drain from the mouth
5. If available, cover casualty with a blanket to keep warm
6. Seek medical attention immediately
7. Document all treatment on DD Form 1380
8. Evacuate as needed

CAUTION: *Do not* give casualty food or drink.

QRC 19.10 HEAD/NECK/SPINAL INJURY

Determine if Care Under Fire, see QRC 19.2 or
Tactical Field Care, see QRC 19.4

Always Treat for Shock, see QRC 19.9

Use Personal Protective Equipment (Gloves)

SYMPTOMS

- Numbness and/or lack of control anywhere below the neck
- Drainage of blood or fluid from the ears, nose, or mouth
- Bruising around eyes (raccoon eyes) or bruising behind the ears (battle signs)

TREATMENT

1. Maintain airway using jaw-thrust maneuver
2. If conscious, have them stay still and monitor their airway
3. Immobilize their cervical spine by holding the head and neck completely still
4. Immobilize the head by using tape, rolled-up towels, or any other effective materials that do not compromise the casualty's airway

If the casualty must be moved:

1. Use a litter (e.g., Talon II®, Sked®, a door, lumber, or other material)
2. Use at least three people to log-roll the casualty while keeping the spine immobilized
3. Turn the body as one unit
4. Secure the casualty to the litter
5. The person at the casualty's head is in charge

QRC 19.11 ABDOMINAL WOUND

Determine if Care Under Fire, see QRC 19.2 or
Tactical Field Care, see QRC 19.4
Always Treat for Shock, see QRC 19.9
Use Personal Protective Equipment (Gloves)

SYMPTOMS

- Complete or partially open wound to abdomen
- Internal organs may be outside of body

TREATMENT

1. If no spinal or leg injuries, bend the knees to relieve pressure on the abdomen
2. If organs are outside body, gently pick them up and place them on top of the abdomen

WARNING: *Do not* replace or push organs back into the body.

3. Cover exposed organs with clean dry dressing

CAUTION: *Do not* wet bandage due to the lack of sterile water or sterile environment in the field.

4. Secure bandages
5. Document all treatment on DD Form 1380
6. Evacuate as needed

QRC 19.12 EYE INJURIES

Determine if Care Under Fire, see QRC 19.2 or
Tactical Field Care, see QRC 19.4

Always Treat for Shock, see QRC 19.9

Use Personal Protective Equipment (Gloves)

SYMPTOMS

- Severe pain or injury to one or both eyes

TREATMENT

CAUTION: *Do not* remove impaled objects.

1. Perform a rapid field test of visual acuity on each eye separately—determine which of the following the casualty can perform:
 - Read print
 - Count fingers
 - Hand motion
 - Light perception
2. Apply rigid eye shield over the injured eye (not a pressure patch)
3. For impaled objects, apply a dressing around the impaled object to secure it in place
4. **Do not** place an eye shield over the uninjured eye to prevent movement; this makes an otherwise ambulatory patient a litter patient and increases the casualty's stress
5. Stay with casualty
6. Document all treatment on DD Form 1380
7. Evacuate as needed

QRC 19.13 SPRAINS/STRAINS/FRACTURES

Determine if Care Under Fire, see QRC 19.2 or
Tactical Field Care, see QRC 19.4

Always Treat for Shock, see QRC 19.9

Use Personal Protective Equipment (Gloves)

SYMPTOMS

- Pain over specific body part
- Bruising
- Swelling
- Deformity
- Numbness/tingling in affected extremity

TREATMENT

CAUTION: *Do not* remove impaled objects.

1. Splint in position found

NOTE: Anything can be used to splint an injury (e.g., sticks, rolled-up papers, blankets).

2. Remove clothing from injured area (if feasible)
3. If able, remove rings from fingers
4. Splint the joints above and below the injury
5. Check for a pulse below the injury site before and after splinting to ensure the splint is not too tight
6. Document all treatment on DD Form 1380
7. Evacuate as needed

QRC 19.14 BURNS

Determine if Care Under Fire, see QRC 19.2 or
Tactical Field Care, see QRC 19.4

Always Treat for Shock, see QRC 19.9

Use Personal Protective Equipment (Gloves)

SYMPTOMS

- Skin discoloration (depending on thickness of burn)
- Pain
- Blisters
- Charring
- Swelling

WARNING: The casualty's airway may have been burned as well

TREATMENT

WARNING: Protect yourself from the source of the burn

1. Stop the burning process (e.g., put out fire, neutralize chemicals, remove from electricity)
2. If able, remove rings from fingers

CAUTION: Do not pop any blisters associated with burns

3. Apply clean, dry dressings and secure with bandages

Note: If the extremities are burned, ensure fingers or toes are loosely and individually bandaged

4. Document all treatment on DD Form 1380
5. Evacuate as needed

QRC 20.1 MEDICAL EVACUATION (MEDEVAC) 9-LINE

LINE 1—Location, give grid coordinates _____

LINE 2—Radio frequency and call sign _____

LINE 3—Number of patients by precedence (if two or more categories must be reported, say “break” between) _____

A—Urgent (Immediate, ASAP)

B—Priority

C—Routine

D—Convenience

LINE 4—Special equipment required _____

A—None

B—Hoist

C—Evacuation equipment (Stokes® litter, etc.)

D—Ventilator

LINE 5—Number of patients by type _____

A—Litter

B—Ambulatory

C—Escort

LINE 6—Security of pickup site (Wartime) _____

N—No enemy troops in area

P—Possible enemy troops in area (approach with caution)

E—Enemy troops in area (approach with caution)

X—Enemy troops in area (armed escort required)

*Number and type of wound, injury, illness (Peacetime)—Specific info (gunshot, shrapnel, bleeding) _____

LINE 7—Method of marking pickup site _____

A—Panels

B—Pyrotechnic signal

C—Smoke signal

D—None

E—Other (mirror, person, light, etc.)

LINE 8—Nationality and status of casualties _____

A—US military

B—US civilian

C—Non-US military

D—Non-US civilian

E—Enemy prisoners of war (EPW)

LINE 9—Status of chemical, biological, radiological, and nuclear contamination (Wartime) _____

C—Chemical

B—Biological

R—Radiological

N—Nuclear

QRC 7.1 WEAPON SAFETY

- Treat all weapons as loaded.
- Never point a weapon at anything you do not intend to shoot.
- Keep your weapon on SAFE until you are ready to fire (unless directed otherwise).
- Keep your finger off the trigger until sights are on target and you are ready to fire.
- Positively identify your target, be aware of what is around and behind the target.
- Never engage in horseplay while handling weapons.

QRC 14.1 5-Cs UXO/IED BATTLE DRILL

5-Cs	Reaction
Confirm	the unexploded explosive ordnance (UXO).
Clear	the area 300 meters (1,000 feet) around the UXO and utilize all frontal and overhead protection and report the UXO to the operations center (see paragraph 14.8).
Cordon	the perimeter around the UXO.
Check	the immediate area for other UXOs. Perform 5/25/Sky meter check.
Control	the site access, maintain security.

QRC 14.2 EOD 9-LINE REPORT

LINE 1—Date-Time Group (when the item was discovered): _____

LINE 2—Report Activity and Location (unit ID and grid location of the IED/UXO): _____

LINE 3—Contact Method:

Radio Frequency: _____

Call Sign: _____

Point of Contact: _____

Telephone Number: _____

LINE 4—TYPE of IED/Ordnance (unknown, dropped, projected, placed, thrown, UAS rotary or fixed wing, or possible IED). Describe the IED/UXO: _____

LINE 5—CBRN Contamination (Negative or Positive)

If present, describe (be as specific as possible): _____

LINE 6—Target/Resources Threatened:

Personnel (coalition forces, host nation, civilian): _____

Facilities: _____

Equipment: _____

Other Assets: _____

LINE 7—Impact of Mission (short description of current tactical situation and how the IED/UXO affects the status of the mission): _____

LINE 8—Protective Measures/Evacuation (measures taken to protect or evacuate personnel and equipment): _____

LINE 9—Recommended Priority:

(Immediate)

(Indirect)

(Minor)

(No Threat)

QRC 14.3 DROPPED ORDNANCE (1 of 2)



Magazine shown for size reference.

QRC 14.3 DROPPED ORDNANCE (2 of 2)



Rifles shown for size reference.

QRC 14.4 PROJECTED ORDNANCE



Rifle(s) shown for size reference.

QRC 14.5 THROWN/PLACED ORDNANCE (1 of 2)



Magazine shown for size reference.

QRC 14.5 THROWN/PLACED ORDNANCE (2 of 2)



Magazine(s) shown for size reference.

QRC 15.1 NINE PRINCIPLES OF IED COMBAT

1. Maintain an offensive mindset.
2. Develop and maintain situational awareness.
3. Remain observant.
4. Avoid setting patterns.
5. Maintain standoff.
6. Maintain 360-degree security.
7. Maintain tactical dispersion.
8. Utilize blast and fragmentation protection.
9. Know and use technology.